

Enlarged Prostate (BPH)

Patient Information · Why urine symptoms happen and how they are treated

WARWIKI

An enlarged prostate is very common as men get older. Its medical name is benign prostatic hyperplasia, or BPH. “Benign” means it is **not** cancer. The larger gland can press on the tube that carries urine and cause bothersome symptoms. There are many good treatments, from simple habit changes to medicines to surgery.

About This Condition

The prostate wraps around the top of the urethra (the tube urine flows through). As the prostate grows, it can squeeze this tube. This causes **lower urinary tract symptoms (LUTS)**:

- A weak or slow stream, trouble starting, straining, or a stream that stops and starts
- A feeling that the bladder is **not empty**, and dribbling at the end
- Going **often** or in a hurry, and waking at night to urinate

Why It Matters

For most men, BPH is about **quality of life**. When it is severe, it can lead to:

- Urine infections or bladder stones
- Blood in the urine
- Being unable to urinate (retention)
- Rarely, strain on the bladder or kidneys

How much your symptoms bother you guides how it is treated.

Learn the Terms

BPH

Benign prostatic hyperplasia. A prostate that has grown larger but is not cancer.

LUTS

Lower urinary tract symptoms. The urine problems BPH can cause.

IPSS

A short symptom score that shows how bothersome things are.

Retention

Being unable to urinate. It needs care right away.

Alpha blocker

A medicine that relaxes the prostate so urine flows more easily.

5-ARI

A medicine that slowly shrinks a large prostate.

PSA

A prostate blood test. It is sometimes checked as part of the visit.

Do I have to treat it? Only if it bothers you, or if problems such as retention or repeated infections appear. Mild symptoms can simply be watched. If symptoms affect your life, treatment can step up from habit changes to medicines to a procedure. The size and shape of your prostate also help your team choose.

How It Is Checked

- A **symptom score** (IPSS) and a review of your drinks and medicines
- A prostate exam and a **urine test**. A **PSA** blood test when it would change your care.
- Often a urine-flow test and a check of how well your bladder empties

How It Is Treated (Step by Step)

- 1 **Watching and habit changes:** drink less in the evening, cut back on caffeine and alcohol, and treat constipation.
- 2 **Medicines:** an **alpha blocker** (works in days to weeks), a **5-ARI** (shrinks a large prostate over months), or both. Other medicines are sometimes used.
- 3 **Less invasive procedures:** such as steam therapy or a urethral lift. Many can be done in the office. They protect ejaculation more often but may need to be repeated.
- 4 **Surgery:** TURP, HoLEP, simple prostatectomy, or Aquablation. TURP, HoLEP and simple prostatectomy open the channel the most and have the longest record of lasting results. Each has its own handout.

Good to Know

- Treatment is about **your** level of bother. There is no rush unless problems appear.
- Some cold and allergy medicines (decongestants and antihistamines) can make symptoms worse.

Call the office that is treating you if you have:

- Blood in the urine
- A temperature of **100.4 °F (38 °C)** or higher, or chills
- Symptoms that get worse despite treatment

Go to an emergency department if:

- You cannot urinate and your bladder feels full or painful
- You cannot reach the office and are worried

Three Things to Remember

1. BPH is a common prostate growth that is not cancer.
2. Treat it based on how much it bothers you.
3. Options range from habit changes to medicines to procedures. Ask about the trade-offs of each.